

Guideline Title: Atrial Fibrillation and Atrial Flutter

Guideline Number: GUID-3203-C003

Issue date: 09/09/2014	Replaces Dept. Policy:
Revision dates: 4/30/24	Developed by: Air Care Team
Approved by: Dr. Christopher Hunter, MD Air Care Team Medical Director	Approved by:
Signature: 	Signature:
Department Numbers: 3203	

- I. **PURPOSE:** Control the ventricular response of the rapid heart rate by stabilizing the abnormal heart rhythm through therapeutic interventions.
- II. **DEFINITIONS:**
 - a. Afib- Atrial Fibrillation
 - b. A-flutter- Atrial Flutter
- III. **DEPARTMENT GUIDELINE:**
 - a. Maintain and manage airway, breathing, and circulation.
 - b. If the patient is not symptomatic with stable vital signs, monitor closely.
 - c. Consider applying hands-free defibrillator pads to patient's bare chest if patient is symptomatic. May proceed directly to cardioversion if patient becomes unstable.
 - d. If patient is hemodynamically unstable (SBP < 90 mmHg, altered level of consciousness, severe chest pain, pulmonary edema):
 - i. Have the patient perform vagal maneuvers while preparing to cardiovert.
 - ii. Perform synchronized *Cardioversion* per GUID3203-PR004. For narrow irregular rhythm, initial recommended dose: 120-200 J Biphasic. Increase subsequent shocks in stepwise fashion.
 - iii. Consider sedation or analgesia per GUID3203-G005 *Analgesia and Sedation Management*.
 - e. If patient is symptomatic and hemodynamically stable, consider the following:
 - i. **Metoprolol 5 mg IV/IO** push over 2 minutes. May repeat x 2 every 5 minutes until a total of 15 mg. Obtain an optimal heart rate of < 120 bpm. ****Contraindicated in patients with congestive heart failure, heart block, valvular failure, or cocaine use****
 - OR-
 - ii. **Diltiazem 0.25 mg/kg (max dose 20 mg) IV/IO** push over 2 minutes. If not effective in 15 minutes, a second dose of **0.35 mg/kg (max dose 25 mg) IV/IO** may be given. ****Contraindicated in patients with congestive heart failure or wide complex rhythm****
 1. If persistent tachycardia above 120 bpm after two boluses, begin **Diltiazem infusion at 5mg/h** and titrate to 15 mg/hr to maintain a HR of < 120 bpm.

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-OR-

- iii. If QRS is wide (greater than 12ms): **Amiodarone 150 mg IV** bolus over 10 minutes followed by **Amiodarone infusion at 1 mg/min.**
- iv. Consider expert consultation.

IV. DOCUMENTATION:

- a. EMS charts

V. REFERENCES:

- a. American Heart Association, 2015.